

Streamlining Hepatitis C Management: A Collaborative Approach Between Clinical Pharmacists and Specialty Pharmacies

Geisinger

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Background/Purpose:

- Geisinger’s Medication Therapy Disease Management (MTDM) program has played a crucial role in initiating and monitoring therapy for patients with hepatitis C since 2016.
- A 99.7% sustained virologic response (SVR) rate has been achieved using collaborative management practices among physicians and pharmacists embedded in the clinics yet care between the clinics and the in-house specialty pharmacy (SP) was redundant and fragmented.
- Integration with in-house SP services aims to streamline care and improve outcomes.

Objectives:

- Describe the co-management workflow of MTDM pharmacists before and after integration with the SP.
- Evaluate the impact of this integration on:
 - Time to treatment initiation
 - Patient adherence
 - Number of patient outreaches

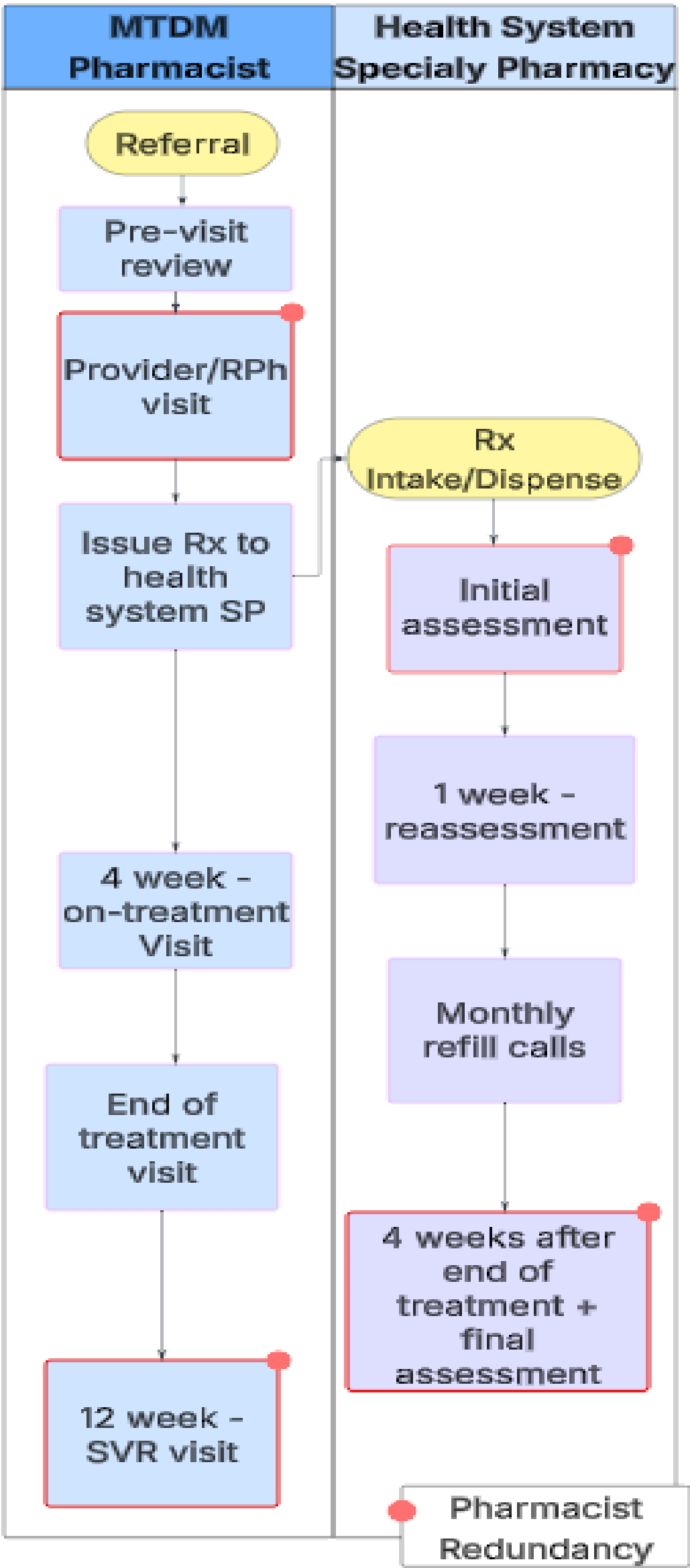
Baseline Metrics :

- Data collected using data from EPIC Hyperspace and Willow Ambulatory

Embedded Hepatology Pharmacists	2
Hep C Specialty Pharmacists	3
Average Start to Treatment (days)	3.42
Average Touchpoints per Treatment	9
Patients Managed	294
Proportion of Days Covered (PDC)	95.64%
SVR Rate	99.7%

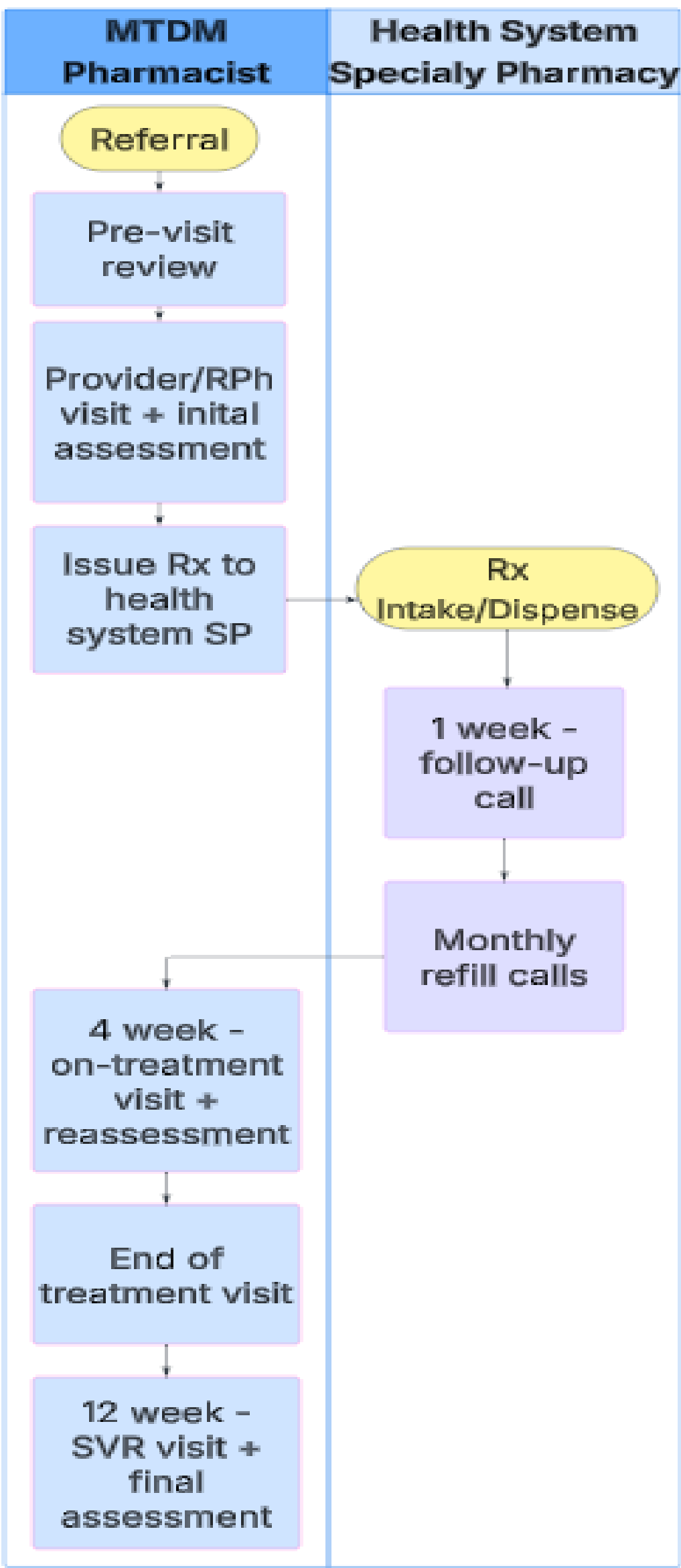
Process:

Baseline:
Independent workflows



Integrated:

Bidirectional communication between teams to reduce duplicative work



Notable Process Changes:

- Initial assessment completed at in-person joint visit with provider and pharmacist
- Health System SP completing 1 week follow-up call to verify patient received medication and started therapy
- Reassessment completed by embedded pharmacist with on-treatment lab visit
- Final assessment completed during visit to confirm SVR and provide end of treatment education

Conclusions:

- Integrating MTDM clinical pharmacist workflows with specialty pharmacy patient management programs is anticipated to streamline care.
- Minimizing delays caused by missed patient communication is expected to increase efficiency, thus resulting in quicker initiation of therapy.
- This integrated clinical management model can be applied broadly across specialties with embedded pharmacists or when designing specialty pharmacy clinical programs.

Next Steps:

- The effectiveness of the integrated model will be assessed by comparing the number of patient outreaches and the time to initiation of treatment.
- Adherence rates before and after implementation will be evaluated to assess the impact of fewer touchpoints.
- Pharmacist engagement will be analyzed after elimination of duplicative tasks.

Acknowledgements:

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